

Executive Summary (UK/US Market Access Divergence in Oncology Biologics: A Trastuzumab Deruxtecan Case Study

Trastuzumab deruxtecan (T-DXd, Enhertu) offers a rare natural experiment in UK/US market access divergence: a single medicine, jointly developed and commercialised by Daiichi Sankyo and AstraZeneca, submitted to both NICE and US payers across two related but distinct indications. With the drug, pricing methodology, and clinical mechanism held constant, the only factor that varies is the regulatory and reimbursement environment the same evidence package is submitted into, making this case unusually well-suited to isolating why UK and US outcomes diverge, not just that they do. The findings below show that the divergence is rooted in how each system generates and weighs evidence, not merely in how each prices it. This distinction carries implications for any two-market launch, not just those involving the UK and US. Four findings, drawn from a verified n=13 NICE appraisal dataset, a four-document theme comparison, and the T-DXd case itself, point to the same conclusion. UK market access risk is structural rather than incidental, and it is most cost-effective to mitigate it during the evidence generation stage of a launch rather than after a US-anchored strategy has already been established.

First, disagreement between a company's cost-effectiveness model and NICE's Evidence Assessment Group (EAG) is the strongest predictor of appraisal outcome observed to date. No appraisal with substantial EAG disagreement was recommended, and none with an aligned assessment was rejected. As the EAG's preliminary view is typically available several months before NICE's binding decision, it acts as an early, internal diligence signal, one sponsors can act on before the pricing and evidence package options it flags have closed.

Second, surrogate endpoint validity (whether progression-free survival reliably predicts overall survival) is an active, recurring point of contest in the NICE appraisals reviewed here, and was categorically absent from the US academic cost-effectiveness literature analysed alongside them. This points to a structural gap rather than a difference in emphasis. An evidence package built to US academic convention risks not generating the long-term survival data a NICE committee independently demands.

Third, in this case, a positive reimbursement outcome did not transfer across indications for the same molecule. T-DXd secured NHS managed access for its HER2-positive indication (roughly 600 eligible patients annually) while its HER2-low indication was rejected outright, with medicine, manufacturer, and appraising body held constant throughout. This suggests each indication requires independent underwriting rather than inheriting a prior approval's track record.

Fourth, the UK-US cost-effectiveness threshold gap is wide and persistent. Even NICE's most generous historical threshold, converted to USD, reaches a maximum of \$66,500 per QALY, still roughly a third less than the US ICER benchmark floor of \$100,000. A global list price

calibrated to US willingness-to-pay carries a significant risk of UK rejection in a way the reverse does not.

A launch strategy built primarily around US regulatory and pricing logic will systematically under-invest in exactly the evidence NICE requires, and will discover the resulting UK access risk only after the decisions that could have addressed it (trial design, evidence package scope, headline pricing) are already fixed.

Combined, these findings show that UK-specific requirements are cheapest to address as a design constraint from the outset and not as a market to revisit after a US-anchored strategy is already set. The analysis is scoped to the UK and US systems specifically. Extending this framework to other health systems would require independent verification of the relevant institutional and legal mechanisms. Full sample sizes, verification methodology, and stated limitations for each finding are set out in the Methodology Appendix.